

Structural Failures of the Punitive Model

A Systems Critique

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Working Paper — October 2026

ABSTRACT

Homelessness policy in the United States has undergone a significant ideological realignment. The Housing First consensus of the 2010s has been systematically displaced by an enforcement-based paradigm driven by model legislation like the Reducing Street Homelessness Act. This framework was adopted across multiple states and codified at the federal level through recent HUD funding reallocations, shifting resources away from permanent supportive housing toward transitional and emergency shelter models. This paper does not evaluate these frameworks on normative grounds; it evaluates them on engineering grounds. The punitive paradigm produces three catastrophic systemic failures. The first is biological: the model deploys legal deterrence against a population whose co-occurring severe mental illness and substance use disorder compromises the volitional capacity that deterrence requires, and the misdemeanor charge and its terminal mechanism, the municipal jail cell, cannot provide medically supervised detoxification, psychiatric stabilization, or nutritional rehabilitation. The second is social: enforcement sweeps destroy the geographically anchored relational networks that constitute the minimal social precondition for survival, and transitional placement models impose artificial tenure limits, typically 12 to 24 months, that prohibit the identity capital accumulation required for durable recovery. The third is financial: the model consumes public capital in recurring running expenditure without producing permanent physical assets, guaranteeing perpetual cost compounding. Each failure is independently sufficient to invalidate the framework. The Systemic Dignity Infrastructure addresses each failure through targeted countermeasures, including a 72-hour Ground Floor Intake Airlock for biological sequencing, Dunbar-scale permanent communities of 150 individuals for relational continuity, and commercial adaptive reuse of surplus office buildings for a financial arbitrage against the municipal baseline. The punitive paradigm's failures are consequences of the model's foundational premises and cannot be remedied within the model's own architecture.

Document Metadata

Keywords

Systemic Dignity Infrastructure, punitive model, Cicero Institute, Reducing Street Homelessness Act, HUD Continuum of Care, biological stabilization, ontological security, financial arbitrage, homelessness policy

JEL Classification

- I38. Government Policy; Provision and Effects of Welfare Programs
- H53. National Government Expenditures and Welfare Programs
- K14. Criminal Law
- R31. Housing Supply and Markets

Target eJournals

- Housing & Urban Development eJournal
- Public Economics eJournal
- Public Policy eJournal

Author Note

This framework is developed as an open-source collaborative infrastructure project. Source files, architectural models, and version history are maintained publicly at: <https://github.com/Systemic-Dignity-Infrastructure/sdi-specification>.

Suggested Citation

DiBella, C.J. (Working Paper, October 2026). *Structural Failures of the Punitive Model: A Systems Critique*. Material Dignity Infrastructure Series. Available at SSRN: <https://ssrn.com/abstract=TBD>

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1. Introduction: The Shift to the Punitive Paradigm

Homelessness policy in the United States has undergone a significant ideological realignment over the past decade. The dominant framework of the 2010s, Housing First, was grounded in evidence demonstrating that unconditional access to permanent housing produced superior clinical and fiscal outcomes compared to treatment-contingent models (DiBella, 2026). By the mid-2020s, that consensus had been systematically displaced. A coordinated legislative campaign, driven in significant part by the Cicero Institute and its model legislation, reoriented state and federal policy toward enforcement-based intervention, encompassing the criminalization of street presence, the replacement of permanent housing with temporary sanctioned encampments, and the imposition of mandatory treatment requirements as conditions of shelter access. This paper does not evaluate these competing frameworks on normative grounds. It evaluates them on engineering grounds. The punitive paradigm, as codified in the Reducing Street Homelessness Act and operationalized through the FY 2026 federal funding apparatus, produces three catastrophic systemic failures, each independently sufficient to invalidate the model, and their simultaneous co-occurrence renders the framework incompatible with the stated objective of reducing chronic street homelessness.

1.1. The Cicero Institute Framework

The Cicero Institute, founded in Austin, Texas in 2016, operates as a policy advocacy organization that drafts model legislation and disseminates it to state legislatures through direct lobbying, political relationships, and publicly available model-bill repositories (The Cicero Institute, 2021). Its principal intervention in homelessness policy is the Reducing Street Homelessness Act, a model bill that reconfigures the state's role from housing provider to enforcement authority. The ideological architecture of the model rests on three sequential claims: that unsheltered individuals retain the voluntary capacity to accept or refuse services and bear individual moral responsibility for their street presence; that Housing First models enable continued substance use and behavioral disorder by removing consequences from self-destructive conduct; and that the appropriate state response is to eliminate the physical viability of street encampments, compelling behavioral change through legal pressure.

Each of these claims rests on a mischaracterization of the physiological and neurological condition of the chronic street population. The preponderance of individuals experiencing chronic homelessness present with co-occurring severe mental illness and substance use disorder, a dual-diagnosis condition that, by clinical definition, compromises executive function, impulse regulation, and forward planning

capacity (DiBella, 2026). The premise that this population retains the full volitional capacity assumed by a deterrence model is a metabolic claim that contradicts clinical evidence, and the Cicero framework, by constructing policy on top of this contested physiological premise, guarantees systemic failure at the point of first contact between the enforcement mechanism and the human body.

1.2. State-Level Adoption

The Cicero model achieved rapid legislative penetration through a strategy of template replication. Texas House Bill 1925, passed by the 87th Legislature in 2021, established the prototype (Texas State Legislature, 2021). The bill classified the act of camping in a public place as a Class C misdemeanor, assigned fine penalties, and, critically, prohibited municipalities from adopting policies that discouraged enforcement of the ban. This last provision was significant in its consequences: it removed local discretion and converted what might have been a variable enforcement practice into a mandatory statewide regime. The mechanism did not criminalize encampment alone; it criminalized municipal non-enforcement of criminalization, and local governments that had developed harm-reduction relationships with their unsheltered populations were legally stripped of that discretion.

Missouri Senate Bill 1106, enacted by the 101st General Assembly in 2022, extended the model's financial architecture (Missouri General Assembly, 2022). Beyond the camping prohibition, SB 1106 redirected state homelessness funding away from permanent supportive housing and toward state-sanctioned temporary encampments with basic utility connections. This was the mechanism's second significant innovation: it did not punish street presence alone. It simultaneously defunded the permanent housing infrastructure that might have resolved street presence, replacing it with temporary facilities designed by their own enabling legislation to be non-permanent. The policy thereby guaranteed the persistence of the population it claimed to address, and by the mid-2020s, analogues of the Cicero model had been enacted in at least sixteen states.

1.3. Federal Integration

The federal codification of the punitive model occurred through the redesign of the U.S. Department of Housing and Urban Development's primary homelessness grant mechanism, the Continuum of Care program. The FY 2026 Continuum of Care Program Competition Notice of Funding Opportunity (Docket No. FR-7000-N-25; CPD-2600-DC-0025), published June 1, 2026, restructured the scoring and eligibility criteria governing approximately \$3 billion in annual federal grants (U.S. Department of Housing and Urban Development, 2026). The redesigned NOFO elevated transitional housing

models, emergency shelter expansion, and treatment-contingent programming in its competitive scoring framework while reducing the scoring weight assigned to permanent supportive housing renewal. The practical consequence was a reallocation of approximately \$1.3 billion away from the permanent housing stock and toward temporary, non-asset-producing interventions, and the federal government did not observe the Cicero-aligned state legislative campaigns from a distance but adopted their fiscal architecture and imposed it on the national grant-making apparatus.

2. The Biological Failure

The first and most fundamental failure of the punitive paradigm is metabolic. The model deploys a legal instrument, criminal misdemeanor classification with its associated fine schedule and incarceration threat, against a human body in physiological crisis. A legal instrument operates on the volitional decision-making capacity of a cognitively intact actor, and the chronic street population, by clinical definition, presents with conditions that compromise that capacity, making the intervention mechanism and its target incompatible at the point of contact.

2.1. The Metabolic Baseline

Chronic street homelessness imposes a documented cascade of physiological degradation. Sustained exposure to outdoor conditions without access to regulated temperature, caloric intake, or hygiene infrastructure produces cumulative metabolic strain across multiple organ systems. Individuals experiencing chronic homelessness exhibit elevated rates of cardiovascular disease, diabetes, respiratory illness, peripheral vascular disease, and infectious disease, conditions that, in the absence of sustained medical management, progressively compromise cognitive and volitional function (DiBella, 2026). Superimposed on this somatic baseline are the neurological sequelae of substance use disorder. Chronic stimulant use produces sustained dopaminergic dysregulation. Chronic opioid use produces physical dependence whose acute withdrawal presents with autonomic instability, severe pain, and gastrointestinal crisis. Chronic alcohol use disorder produces thiamine deficiency with potential progression to Wernicke encephalopathy, an acute neurological emergency, and, in withdrawal, the risk of fatal seizure.

These are physiological states that require medical management, and the punitive model's failure to account for this metabolic baseline constitutes a design defect. The application of a misdemeanor charge to an individual in acute dual-diagnosis crisis does not resolve the metabolic crisis; it adds a legal liability to an already degraded physiological state. The downstream consequences are

well-characterized: criminal records reduce future employment eligibility, restrict access to certain housing programs, and add administrative complexity to the acquisition of government benefits. The misdemeanor charge produces a net reduction in the individual's capacity to achieve the stable housing that would resolve the street presence the charge was designed to address, and the punitive mechanism thereby degrades the very preconditions required for its stated outcome.

2.2. The Fallacy of the Jail Cell

When a fine cannot be paid, a condition guaranteed in a population without income, the enforcement pathway terminates in incarceration. The municipal jail cell is the punitive model's terminal resolution mechanism, presented implicitly as a stabilizing intervention that removes the individual from the street, provides shelter, and creates an opportunity for behavioral change. This characterization fails on clinical grounds. A municipal jail cell is not a clinical facility. It does not provide the medically supervised detoxification required to safely manage withdrawal from alcohol or opioids, and it does not provide the psychiatric stabilization required for acute psychotic episodes, or the nutritional rehabilitation required for thiamine-deficiency reversal.

It provides physical enclosure and behavioral restriction, conditions that, in the absence of concurrent medical management, allow acute physiological crises to persist or worsen. The individual released from a 30-day incarceration period has experienced enforced abstinence in a medically unsupervised environment, has lost whatever street-level support networks allowed survival, and faces the transition back to the street with a criminal record that further constricts access to formal support systems. The jail cell, as an intervention mechanism for metabolic crisis, produces net harm.

2.3. The SDI Resolution

The Systemic Dignity Infrastructure resolves the biological failure by establishing a sequencing requirement that the punitive model cannot satisfy (DiBella, 2026). Physical stabilization must precede all other interventions; this is an engineering constraint, not a preference. An individual in active metabolic crisis cannot engage meaningfully with housing intake processes, case management appointments, employment training, or social network reconstruction, and every intervention that attempts to address the social or economic dimensions of homelessness before addressing the physiological baseline is operating against the causal chain of the problem.

The SDI architecture operationalizes this sequencing requirement through the Ground Floor Intake Airlock, a dedicated medical processing node positioned at the building's street-level entry point. Every incoming resident passes through a 72-hour monitoring period in which vital signs, acute

metabolic indicators, and substance use status are assessed and stabilized before the individual is assigned to a permanent residential unit. This is the gating condition that governs all subsequent intervention, and an individual who cannot safely inhabit a residential unit, because they are in acute alcohol withdrawal, experiencing a psychotic episode, or presenting with untreated sepsis, is routed to the appropriate clinical facility before unit assignment occurs. The airlock does not incarcerate or penalize; it performs the metabolic triage that any functional intake pipeline requires.

3. The Social Failure

The second failure of the punitive paradigm operates at the social layer. Chronic homelessness is not a housing deficit alone; it is a condition of sustained relational impoverishment, the progressive erosion of the social networks, identity anchors, and institutional connections that enable functioning within civic society. The punitive model, through its enforcement sweeps, forced displacement, and temporary placement architecture, actively destroys the residual social infrastructure that individuals have constructed under conditions of extreme adversity, and this destruction is a direct consequence of the model's core mechanisms, not an incidental byproduct.

3.1. Relational Destruction

Individuals experiencing long-term street homelessness develop, over time, place-specific social networks of considerable functional significance. These networks provide mutual monitoring for medical emergencies, reciprocal resource sharing, informal security, and the basic relational recognition, being known by name and having one's history acknowledged, that constitutes the minimal social precondition for psychological stability (DiBella, 2026). These networks are geographically anchored; they depend on spatial co-location maintained over time and cannot be transferred to a new location or reconstructed on demand.

Enforcement sweeps under the Cicero model are designed to disaggregate these spatial concentrations. The legal mechanism, misdemeanor citation and camp dispersal, physically separates individuals who have developed stable relational bonds. This disaggregation is the mechanism's stated purpose: the policy goal is to render street encampment sufficiently costly and unstable that individuals will accept placement in state-sanctioned facilities. The implicit assumption is that the relational networks anchoring individuals to encampment sites are obstacles to housing placement, a characterization that is clinically incorrect. The destruction of these networks removes the social scaffolding that allows individuals to tolerate the stress of transition, complete intake processes, and maintain the

behavioral consistency required for sustained tenancy, and enforcement sweeps, by this analysis, degrade the social preconditions required for placement to succeed.

3.2. The Artificial Horizon

The punitive model's alternative to street encampment is the state-sanctioned temporary facility, a time-limited placement with an enforced discharge date. Transitional housing models, including the sanctioned encampments authorized under Texas HB 1925 (Texas State Legislature, 2021) and Missouri SB 1106 (Missouri General Assembly, 2022), impose artificial tenure limits, typically in the range of 12 to 24 months. These limits are features of the funding model: temporary facilities are funded on ongoing running cost terms rather than capital terms, and their per-unit costs are justified by turnover rates. A facility that retains residents indefinitely fails its financial model, so the discharge date is a fiscal constraint imposed on a clinical population, not a clinical determination.

The consequences are predictable. An individual who has spent 24 months in a temporary facility has experienced a period of relative stability, regular meals, physical shelter, and some access to case management services, followed by a mandated return to the conditions that produced street homelessness in the first instance. The social networks they maintained before placement have been disrupted by their absence, and the networks they may have formed within the temporary facility dissolve at discharge. They return to the street with fewer social connections than they had before placement, and the revolving door actively degrades the social capital that resolution would require, so that each cycle through the system produces a net reduction in the individual's relational resources.

This pattern maps directly onto the theoretical construct of identity capital deficit developed within the foundational theory of the Systemic Dignity Infrastructure (DiBella, 2026). Identity capital, the accumulated portfolio of social roles, institutional connections, relationships, and self-narrative coherence that enables an individual to function as a recognized social actor, cannot be constructed in a 24-month temporary placement; it requires the temporal continuity and relational permanence that only a permanent, unconditioned housing environment can provide. The artificial horizon imposed by the transitional model is a prohibition on identity capital accumulation.

3.3. The SDI Resolution

The Systemic Dignity Infrastructure resolves the social failure through two features: the permanence of tenure and the Dunbar-scale ceiling on community size (DiBella, 2026).

Tenure permanence eliminates the artificial horizon. A resident's occupancy of their Asset Limited Modular Unit is not conditioned on behavioral compliance, treatment participation, sobriety maintenance, or employment status. The physical lease is severed from every clinical and behavioral milestone, and eviction requires only verified physical assault or kinetic destruction of the capital property. This unconditional baseline creates the temporal continuity that identity capital accumulation requires. A resident who remains in the same unit, within the same community, for five or ten years has accumulated a history, a set of relationships, and a recognized position within a stable social environment, and that accumulation is the mechanism of recovery.

The Dunbar ceiling of 150 individuals per residential pod operationalizes the relational architecture required to make permanence functional. Anthropological evidence, consolidated in the Dunbar coefficient framework, identifies 150 as the approximate upper bound of stable social groups in which each member can maintain a meaningful personal relationship with every other member (DiBella, 2026). Beyond this threshold, the cognitive demands of relationship maintenance exceed human capacity, and the social group begins to stratify into anonymous subgroups governed by institutional norms rather than relational ones. At or below 150, a residential community retains the social density required for peer monitoring, mutual accountability, informal support, and the ambient relational recognition that Giddens identifies as the foundational precondition for ontological security. The SDI architecture enforces this ceiling as an absolute engineering constraint, and pod populations exceeding 150, counting both residents and dedicated staff, trigger mandatory architectural segregation into independent sectors.

4. The Financial Failure

The third failure of the punitive paradigm is fiscal. The model deploys public capital, state and federal funds, into interventions that produce no permanent physical assets, carry high recurring running costs, and yield no measurable reduction in the population requiring intervention. The result is a pattern of sustained capital evaporation: public funds consumed without producing the durable infrastructure that would reduce future expenditure. This pattern is not a feature of inadequate funding levels; it is a consequence of the model's design, and no level of additional investment in the punitive framework will produce asset accumulation because the framework's mechanisms are explicitly temporary.

4.1. Evaporating Capital

The direct running costs of the punitive model are substantial and recurring. Municipal camping enforcement requires police personnel, equipment, administrative processing, and judicial resources for each citation and arrest. A single enforcement sweep of a large encampment may require dozens of officer-hours. The costs are incurred every time the sweep occurs. Because the enforcement mechanism does not resolve the underlying condition, because no housing is produced, no metabolic stabilization is achieved, and no relational infrastructure is constructed, the same population remains on the street and the sweep must be repeated. The running costs compound without limit because the condition that drives them is never resolved.

Temporary sanctioned encampments produce a second category of recurring cost. They must be staffed, supplied, maintained, and administered continuously. Unlike permanent housing, they accumulate no equity and produce no asset that could support future activity. Their capital expenditure, site preparation, utility connection, and facility construction, is sunk into infrastructure with a defined lifetime. When the facility closes or the population is dispersed, the capital investment produces nothing recoverable. The funds are consumed and gone. This pattern replicates indefinitely across the program cycle because the temporary nature of the facilities guarantees their eventual closure and the repetition of site preparation expenditure.

4.2. The \$1.3 Billion Deficit

The federal reallocation codified in the FY 2026 HUD Continuum of Care NOFO represents the quantification of this capital evaporation at scale (U.S. Department of Housing and Urban Development, 2026). The shift of approximately \$1.3 billion away from permanent supportive housing, a capital-productive intervention that builds stable housing stock with ongoing value, toward transitional models and emergency shelter represents a system-level decision to substitute recurring running cost for one-time capital investment. The long-run fiscal consequence is deterministic. Permanent supportive housing produces a durable physical asset, and once constructed, a permanent supportive housing unit provides ongoing housing capacity at marginal running cost. An emergency shelter produces no such asset and must be funded continuously to provide the same capacity.

The fiscal comparison is not between the annual running costs of permanent housing and the annual running costs of temporary shelter. It is between the annualized total cost of permanent housing, capital amortization plus running cost, and the perpetual total cost of temporary shelter, which carries no amortization because no capital asset is produced. Over any planning horizon exceeding

the amortization period of permanent housing stock, the temporary model is the more expensive option. The FY 2026 NOFO reallocation represents the deferral of capital investment in exchange for guaranteed perpetual running expenditure.

4.3. The SDI Resolution

The Systemic Dignity Infrastructure resolves the financial failure by restructuring the capital mechanism (DiBella, 2026). The architecture bypasses the residential real estate market entirely, targeting instead the surplus commercial real estate stock that has accumulated in major metropolitan areas as a consequence of post-pandemic office vacancy. Commercial adaptive reuse allows the acquisition of large-footprint buildings at substantial discounts to residential construction cost, discounts that, properly structured, produce a per-resident acquisition cost low enough to support a financially viable arbitrage against the municipal spending baseline.

The arbitrage is quantified as follows. The municipal baseline cost of managing an individual experiencing chronic homelessness, aggregating emergency room utilization, jail incarceration, emergency shelter cycling, law enforcement interaction, and social service administration, is documented in the range of \$40,000 to \$80,000 annually depending on jurisdiction and methodology, with a conservative cross-system estimate of approximately \$50,000 per person per year (DiBella, 2026). The SDI architecture targets a fully loaded per-resident annual cost of \$24,000 to \$29,000, representing a 42 to 52 percent reduction against this baseline. This target is composed of two components: an annual running cost of approximately \$18,000 per resident, and an annualized capital service cost derived from the per-resident acquisition cost of approximately \$106,034, amortized over a 30-year bond-equivalent term at 4 percent, yielding an annual debt service of approximately \$6,132 per resident. The total annualized cost of \$24,132 per resident represents a 51.7 percent reduction against the municipal baseline.

The mechanism that makes this arbitrage possible is asset permanence. The SDI tower converts capital expenditure into a durable physical asset that continues to provide housing capacity after amortization, and once the acquisition cost is retired, the per-resident carrying cost drops to the running component alone, approximately \$18,000 annually, representing a 64 percent reduction against the municipal baseline. The financial logic of the SDI architecture becomes progressively stronger over time, as amortized assets produce housing capacity at declining marginal cost while the punitive model's running expenditures continue to compound without limit.

5. Conclusion

5.1. The Systemic Incompatibility

The punitive paradigm, as codified in the Reducing Street Homelessness Act model legislation (The Cicero Institute, 2021), operationalized through state enactments including Texas HB 1925 (Texas State Legislature, 2021) and Missouri SB 1106 (Missouri General Assembly, 2022), and embedded in federal grant architecture through the FY 2026 HUD Continuum of Care NOFO (U.S. Department of Housing and Urban Development, 2026), fails simultaneously at three independent levels. Biologically, it deploys legal deterrence against a population whose condition compromises the volitional capacity that deterrence requires, and routes individuals experiencing metabolic crisis through incarceration environments that lack the clinical infrastructure to address that crisis. Socially, it destroys the street-level relational networks that constitute the minimal social precondition for survival, imposes artificial tenure limits that prohibit identity capital accumulation, and returns individuals to street conditions with degraded social resources after each cycle. Financially, it consumes public capital in recurring running expenditure without producing permanent physical assets, and the federal reallocation of approximately \$1.3 billion toward these non-capital-productive mechanisms guarantees that this evaporation will continue at scale.

These three failures are consequences of the model's foundational premises, not independent policy deficiencies correctable by targeted modifications. A framework premised on individual volitional responsibility as the lever of behavioral change cannot address a population defined by compromised volition. A framework premised on temporary placement as the mechanism of transition cannot produce the relational permanence required for recovery. A framework premised on running expenditure as the primary capital instrument cannot produce asset accumulation. The failures are built into the architecture.

5.2. The Threshold for Permanence

Systemic stabilization of the chronic street population requires permanent physical and relational infrastructure. This is an engineering specification derived from the causal chain of the problem. Metabolic stabilization requires a clinical environment capable of medical management. Relational recovery requires a social environment stable enough and permanent enough to support identity capital accumulation over multi-year timescales. Financial sustainability requires capital asset production rather than recurring running expenditure. A temporary mechanism that cannot satisfy

these requirements is a management protocol for perpetuating the condition it purports to resolve, not an intervention in it.

The Systemic Dignity Infrastructure specifies the minimum conditions under which a permanent solution becomes viable: a 72-hour medical airlock for biological sequencing, a Dunbar-scale permanent community for relational continuity, and a commercial adaptive reuse capital mechanism for financial sustainability (DiBella, 2026). These are engineering requirements. The Singular Prototype, the minimum viable implementation of the complete architecture, must satisfy seven binary verification metrics before network expansion proceeds, and any evaluation of the SDI model must be made against these specifications, not against the unverified claims of models that have demonstrated only the capacity to perpetuate the conditions they purport to resolve.

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